

Smile Club — Membership Growth Plan

Rev. 2, incorporating external review · Prepared by Fahad Siddiqui, Growth · 15 September 2026

Review copy for **Gautam** — the live version runs on the Growth Projects dashboard; this document mirrors it in full.

The recommendation: proceed with a focused, measurable pilot; expand when the evidence supports it. Start with existing patients and clinic touchpoints, validate the offer and its economics, run one clearly defined corporate pilot through a warm door, and defer expensive awareness media. Scale is a decision we earn with data at the end of the pilot — not a calendar commitment we make today.

Gautam — what the plan needs from you, specifically

1. **The baseline (Day-3 item — everything starts here).** Current enrolments, payment status (active / failed / cancelled), sign-up dates, plan mix (Essential vs Plus, monthly vs annual), benefit usage, cancellations. Without it every result reads as “up from unknown”.
2. **Activation tracking.** We measure two separate events per member: first appropriate *booking* and first *completed* appropriate visit, with defined cohort windows. Please review whether the Smile Club console can report both today, and what is missing if not.
3. **Onboarding journey (Wave 1, with front desk).** Welcome + clear benefits/terms; help arranging the first appropriate appointment; payment-failure follow-up; cancellation reasons captured; renewal preparation.
4. **Referral mechanic (build as a test).** Member-get-member with tracking — referral is a channel to test, not an assumed winner.
5. **Retention measures.** Monthly-payment persistence and renewal by cohort — two different measures, never conflated.
6. **Monthly Smile Club performance report** — restarts once the baseline lands.

Your review question for the whole document: is anything here impossible, mis-defined, or easier than we think, given how the Smile Club console actually works?

What this document contains

- The diagnosis — five hypotheses to test, and what is fact vs assumption (p.2)
- Offer validation and the economics gate — savings math, contribution, capacity (p.3)
- Three waves, evidence-gated — who does what (p.4)
- The corporate model, measurement definitions, and the first fortnight (p.5)

We have distribution assets; **qualified reach and conversion remain to be measured**. Existing patients may trust the brand and still find the membership poor value, unaffordable, confusing, inconvenient, or hard to use. Affinity, value, affordability, clarity and convenience are all candidate constraints — the pilot is designed to tell them apart. During the pilot: a small sample of member and non-buyer interviews, plus structured objection logging at the front desk and in CRM replies.

The sequence — existing patients → proof → corporate — remains the starting strategy: the sensible first route, not the only possible one.

What is fact, what is reported, what is unknown

CATEGORY	ITEMS
Established (our own platform)	Smile Club product + admin console live · 4.9★ across 60 publicly visible Google reviews (verified 14 Sep) · patient audience lists exist in both ad accounts · warm corporate contacts (Michael Page HR contact, RBS, current partners)
Source-reported (ZAVIS — not independently verified)	25 WhatsApp segments with automated creative generation · 19,500+ programmatic SEO pages · Patient Intelligence ↔ Content OS API integration
Public-page observations (12 Sep — recheck before use)	Essential listed at AED 99/month or AED 999/year · Plus at AED 139/month or AED 1,399/year · displayed “annual savings” compare payment frequencies, not savings vs buying care separately · several FAQs refer readers to the Member Terms. Content review only — not a checkout or functionality audit.
Data still needed	Member baseline — owner: Gautam · clinic prices and benefit-delivery costs (owner: finance/ops, to confirm) · page-level traffic and conversion data for placement decisions

The chicken-and-egg, and how the sequence breaks it. Corporate sales need brand proof (“why Dental Nation?” is HR’s fair question) — and proof needs members. The breaker: start where trust already exists (our own patients), convert usage and savings into a proof pack, then sell corporate through warm doors. Corporate *discovery* conversations start in parallel from day one; only the broad outbound push waits for evidence.

The savings math — with actual prices, not assumptions

$$\text{Net customer saving} = \text{usual price of the same expected, clinically appropriate care without membership} - \text{membership fee} - \text{remaining charges for that care with membership}$$

- Distinguish annual-vs-monthly *billing* savings from savings versus buying care separately — the public page currently displays the former.
- Do not assume every included benefit is needed, used, or always available.
- Individual and family examples built once actual price inputs are confirmed — no invented figures.
- Terms understandable before purchase: inclusions, exclusions, frequencies, availability, discount-combination rules, minimum commitment, cancellation, renewal, insurance interaction.
- Distinct from insurance — positioned against *verified* coverage gaps for the specific audience.

The expansion gate — six economics rules

RULE	WHAT IT MEANS IN PRACTICE
Contribution, not revenue	Membership contribution after the cost of delivering included care, discounts, payment processing, support, refunds and acquisition. If treatment revenue is already net of discounts, they are not deducted twice.
Incrementality	Does the offer drive additional profitable visits, or mainly discount care patients would have bought anyway? Comparison group where practical; platform attribution alone does not prove incrementality.
True channel costs	Staff time, creative review, messaging and event delivery counted. Existing tools reduce production effort; they do not make execution free.
Allowable CAC from economics	The acquisition-cost ceiling and payback criterion come from measured economics — no universal CAC or LTV target is invented.
Cash vs profit	Annual payments are cash received against future care obligations — not automatically profit available for expansion.
Capacity as a gate	Branch appointment availability, staff workload and delivery capacity are explicit expansion conditions — selling memberships we cannot serve destroys renewals and the brand.

Advertising-audience rule (corrected from the first draft): a documented eligibility check precedes any patient-list ad targeting — Google restricts advertiser-curated audiences for sensitive health categories, and Meta’s customer-list terms require the necessary rights and lawful basis. CRM broadcasts require confirmed contact permissions and opt-out handling. Where audience use is unsuitable: eligible search-intent or geographic tests instead.

Expansion at each step depends on: reliable measurement and clear offer terms · evidence of conversion and appropriate activation · acceptable acquisition cost and expected contribution · sufficient appointment capacity and acceptable member experience. A wave funds the next only after considering contribution, cash requirements and future benefit obligations.

Wave 1 — Prepare, then a limited patient test (first fortnight · low incremental media spend; staff and delivery costs tracked)

PLAY	OWNER	DETAIL
Baseline & terms first	Gautam + finance/ops	Enrolments, payment status, plan mix, usage, cancellations; delivery costs; prices; terms. Nothing scales before this exists.
Limited CRM test	Fahad + ZAVIS	A few appropriate, consented segments with a comparable uncontacted group where practical — denominators and an incrementality read.
Front-desk route	Front desk + Dr Luvi	One consistent explanation, simple enrolment, QR at three branches; objection log from day one.
Relevant placements only	ZAVIS + W3Layouts	Placements on pages with demonstrated traffic and local intent — page count alone is not opportunity.
Onboarding & activation	Fahad + Gautam + front desk	Welcome + terms; first-appointment help; first booking and first completed visit tracked separately; reminders; payment-failure follow-up; member value statement.
Corporate discovery (parallel)	Fahad + Mr Akbar	Warm HR conversations start now — discovery does not wait for a case study.

Wave 2 — Scale what the evidence supports (indicative weeks 3–8)

PLAY	OWNER	DETAIL
Paid acquisition tests	Fahad	After the audience-eligibility check; search-intent and geo tests first; offer-led CTWA and Google Search on cost/offer intent.
SEO membership cluster	ZAVIS	EN/AR membership pages + membership module on the highest-traffic cost and treatment pages.
One defined corporate pilot	Fahad + Mr Akbar	One employer, explicit commitments and economics (spec on p.5).
Creative production	ZAVIS	Automation supports assets; offer accuracy, clinical review and performance still need validation before scale.
Referral mechanic — test	Fahad + Gautam	Member-get-member built and measured; a channel to test, not an assumed winner.

Wave 3 — Compound, if the gate opens (quarter +)

PLAY	OWNER	DETAIL
Corporate expansion	Fahad + Mr Akbar	Outbound to a named employer list opened with the pilot case study; benefits platforms, chambers, HR communities.
Insurance-gap positioning	Fahad	Against verified dental-coverage gaps for the specific audience.
LinkedIn corporate lane	ZAVIS	Corporate-benefits content aimed at HR titles, warming the outbound list.
Selective offline	Fahad	Judged case by case on fit, total delivery cost, attributable enrolments. Radio and billboards stay deferred.
Retention economics	Fahad + Gautam	Persistence and renewal by cohort — not validated by a short pilot; cohorts must mature.

Three corporate models — never blurred

1. **Employee-paid**, distributed through the employer. A discount code is a distribution mechanism — not evidence of bulk sales. Cost-free to the employer.
2. **Employer-subsidized**. Cost depends on the subsidy.
3. **Employer-paid**. The only true bulk sale — reported as contracts.

Vocabulary: **eligible employees** → **paid enrolments** → **activated members** (+ registered beneficiaries on family plans). The promise is *low administration with responsibilities agreed upfront* — not “zero admin”. Reporting to employers is aggregated only, never individual treatment information; testimonials and logos only with permission.

Each pilot specifies

- Employer profile + geographic fit · eligible employee count + payment model
- Named sponsor + communication commitments · duration, offer, onboarding
- On-site day scope, staffing and cost (estimated per event) · tracking code + attribution
- Success criteria + review date · reached → enrolled → activated, with cost and contribution

Measurement — defined denominators

MEASURE	DEFINITION
Memberships vs members	Contracts separated from people; cancellations and failed payments defined.
Conversion	Paid enrolments ÷ an explicitly defined eligible or exposed audience, by channel.
CAC	Consistent allocation ÷ new paid memberships; attributed vs incremental distinguished.
Activation	First booking AND first completed visit, cohort-windowed. A two-week pilot cannot establish 60-day activation or renewal.
Contribution	Net of delivery + operating costs; annual fees = future care obligations.
Incremental	Vs what the same audience would have done without the intervention.
Capacity	Appointment availability, complaints, promised service levels.
Retention	Payment persistence AND renewal by cohort — two measures.

Where inputs are missing: “*baseline pending*” or “*target to be set from baseline and economics*”. Utilization is encouraged only where clinically appropriate — never to improve a metric.

The first fortnight

DUE	DELIVERABLE	OWNER
Day 3	Baseline: enrolments, payment status, sign-up dates, plan mix, usage, cancellations; traffic/conversion data	Gautam (data) · Fahad (traffic)
Day 5	Delivery costs, clinic prices, capacity, terms confirmed; realistic savings examples built	Finance/ops — owner to confirm
Day 5	Funnel, tracking, cohort definitions, objection log, pilot budget, expansion criteria defined	Fahad
Day 8	Front-desk explanation, targeted CRM messages, placements, onboarding journey prepared	Fahad + ZAVIS + front desk
Day 10	Limited patient test live with comparison group where practical; conversion, objections, bookings, capacity monitored	Fahad + ZAVIS
Day 12	Warm corporate discovery held; one pilot defined with explicit commitments and economics	Fahad + Mr Akbar
Day 14	Fortnight review: fix, continue testing, or expand provisionally	Fahad → Mr Akbar